Weekly paclitaxel + pertuzumab + trastuzumab

Paclitaxel on Days 1, 8 and 15 of each 21-day block, for up to eight cycles; IV pertuzumab and trastuzumab on Day 1. This NCCP alternative applies when docetaxel is unsuitable, not tolerated or caused significant toxicity. Antibodies may continue after the taxane stops while beneficial and tolerated.

Source reconciled; independent clinical, pharmacy and patient review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart

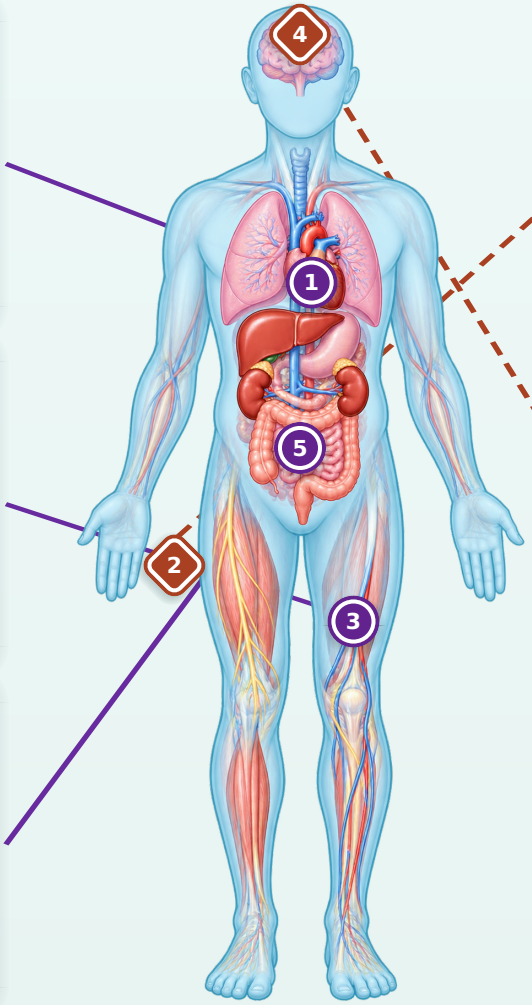
Trastuzumab / pertuzumab

Keep echo appointments; report breathlessness or swelling.

2 Hands / feet

Paclitaxel

Report numbness or tingling before the next dose.



3 Blood / infection

Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

4 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.

5 Bowel

Pertuzumab / chemotherapy

Report diarrhoea promptly, especially with poor intake.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Markers refer to the named active medicines. During HER2-only continuation, chemotherapy effects are not new expected effects of the antibodies; lingering chemotherapy effects still need review.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Paclitaxel

- Low blood counts and infection
- Numbness / tingling, sometimes lasting
- Hair loss, muscle or joint pain
- Infusion reaction
- Mouth, bowel, skin and nail effects

Pertuzumab

- Diarrhoea and dehydration
- Infusion / allergic reaction
- Heart pumping-function reduction
- Rash and fatigue

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

The correct phase prescription

Taxanes use low-risk antiemetic support; hypersensitivity premedication is a separate requirement.

Premedicines and growth factors

Paclitaxel premedicines differ between weekly and fortnightly dosing. Use the phase-specific prescription.

Tests and appointments

The team should name the booking owner, due window and person reviewing each required test. Tell them if an expected appointment has not arrived.

After chemotherapy

HER2-only continuation has its own minimal-emetic-risk plan; do not automatically continue chemotherapy steroids, antiemetics or growth factors.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

CUH · MON–FRI 8AM–5PM

087 181 9645

Acute Oncology Service

CUH · WEEKENDS / BANK HOLIDAYS 9AM–5PM

021 4922000

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00507/

Sources and review status are available online. Follow your own prescription and hospital instructions.